



Guideline

Mental Health Missing Persons/Absent Without Leave

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Version	2.0
Functional subgroup	Mental Health Service
Summary	This guideline outlines the processes to be followed for risk assessment and risk mitigation to minimise the risk of people accessing EMHS mental health services going missing or Absent Without Leave from an inpatient unit, a community mental health service, general hospital, or Emergency Department, as well as the reporting and investigation obligations when such incidents occur.
Policy owner	Executive Director of Mental Health
Contact	Executive Director of Mental Health
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NSQHS Standards (second edition)	Standard 1: Clinical Governance Standard 5: Comprehensive Care Standard 8: Recognising and Responding to Acute Deterioration
Impact Statement Declaration (ISD)	The consultation through the Aboriginal community is reflected in this policy to ensure cultural appropriateness and positive health impacts on the Aboriginal community has been included (ISD Record ID)
Status	Active
Information Classification	OFFICAL

1. Background

1.1 About this document

This Guideline provides guidance to support the implementation of the Mental Health Missing Persons/Absent Without Leave Policy within EMHS mental health services.

This Guideline is to be read in conjunction with the Mental Health Missing Persons/Absent Without Leave Policy which provides clear direction and actions required where a person accessing care is missing/AWOL as defined by section 1.2 of this document.

Note It is important to note that the definition for a missing person according to the Chief Psychiatrist differs from the definition used by the WA Police Force (WAPOL).

For the purposes of this guideline, any person receiving mental health care whose whereabouts are unknown and who is at risk of harm must be reported to WAPOL.

1.2 Key definitions

Term	Definition
Absent Without Leave (AWOL)	Under section 97 of the Mental Health Act 2014 (WA) (MHA), a person is considered Absent Without Leave (AWOL) from an authorised hospital or other place (such as a clinic) if they leave without permission, when they are: 1) detained under the MHA: a. to be taken (transported) to an authorised hospital or other place (ie. the person is under Form 4A); or b. for assessment or examination (ie. once the person is received in the authorised hospital or other place and are under Form 1A or 3A); or 2) under an inpatient treatment order (ie. the person becomes an involuntary inpatient and under Form 6A) and: a. leave has not been granted; or b. they fail to return to the hospital after their leave has been cancelled or expired; or 3) an involuntary community patient who is ordered to attend a place as a result of breaching their community treatment order (CTO) (ie. person is under Form 5F) and is required to be detained at that place (see s 130(2)(b) MHA).

Missing person	The Office of the Chief Psychiatrist Policy for Mandatory Reporting of Notifiable Incidents to the Chief Psychiatrist – Policy for Public Mental Health Services defines a ‘missing person’ as “any voluntary psychiatric patient with risk concerns for harm, who is missing from a mental health service, general hospital or emergency department, without the agreement of or authorisation by staff.”
Harm	As per the Department of Health’s Mental Health Consumers Who Are Missing or Absent Without Leave Policy and the Office of the Chief Psychiatrist Policy for Mandatory Reporting of Notifiable Incidents to the Chief Psychiatrist – Policy for Public Mental Health Services , harm is defined as: harm to self, harm to others, harm from others, and harm to property, such as arson.

Refer to:

- [Chief Psychiatrist Clinicians’ Practice Guide to the Mental Health Act 2014](#) (section 4.10)
- [WA Mental Health Act 2014](#)

2. Risk Assessment

When to conduct risk assessments:

Early risk assessment and regular reassessment must be completed to determine the risk of a person accessing mental health services going missing or AWOL, particularly at significant points in care such as:

- At activation/admission
- Prior to change of setting or transfer of care
- Following a change in clinical status
- Prior to, and on return from, new leave episodes
- At discharge/deactivation

When conducting risk assessments, the following factors must be considered:

Research has shown that many individuals who become missing or AWOL give the following reasons:

- Boredom
- Fear of other people receiving care in the inpatient unit
- Lack of privacy
- Lack of meaningful recreation and leisure
- Lack of insight into the need for hospitalisation
- Feelings of confinement (which may be related to the quality of the care environment e.g. overcrowding)

- Perceived household responsibility e.g. need to get home to finish tasks or take care of a pet – tasks that are perceived to be more important than their care
- Isolation from relatives and friends
- Cultural reasons, e.g., sorry business (death/funerals)

People who have a greater risk of going missing or AWOL include:

- Those who have previously gone missing or AWOL
- Young people, especially young males
- Young people who are receiving care in an adult mental health service
- People who are culturally and linguistically diverse (CALD)
- Aboriginal and Torres Strait Islander people
- Refugees
- Older adults

Other risk factors that may contribute to the risk of people going missing or AWOL include:

- Diagnosis of a moderate to severe mental health or behavioural disorder
- Alcohol and other substance use
- Dementia / delirium and associated wandering behaviour
- Self-harm behaviours and/or suicidal intent
- Expressed intent to harm others

When a person receiving care is missing or AWOL, the risk of the below may increase:

- Self-harm or suicide
- Deterioration of mental and physical health
- Harm to others or property
- Exposure to unsafe situations

Risk documentation:

All identified risks are to be documented in the Risk Assessment and Management Plan (RAMP). Health professionals in non-mental health sites such as Emergency Departments may use the appropriate equivalent documentation. Refer to relevant local, service guidance/policies.

Inpatient staff should be aware of the levels of risk of going missing or AWOL for all people receiving care and any specific observation requirements for each person. Alerts are to be placed in healthcare records and relevant systems such as PSOLIS and WebPAS for all people receiving care with a history of going missing or AWOL. Refer to:

- MP 0155/21 [Statewide Standardised Clinical Documentation for Mental Health Services](#)

- [Triage to Discharge - Mental Health Framework for State-wide Standardised Clinical Documentation](#)

3. Risk Mitigation

Person-centred safety plans are to include individualised risk mitigation strategies based on best practice to minimise the occurrence of people becoming AWOL or missing. Safety planning is to be documented and updated as part of a person's Treatment Support and Discharge Plan.

Risk mitigation strategies to be used for all people receiving care must include:

- Collaborating with all people receiving care and their support people (as appropriate) in:
 - identifying risks of going missing or AWOL; and
 - safety planning and updating their safety plan at every stage of care including transfer of care and change in clinical presentation.
- Incorporating person-centred, recovery-oriented, culturally appropriate, trauma-informed and developmentally appropriate approaches into care and safety planning.
- When an Aboriginal person is identified as at risk of going missing or AWOL, Aboriginal Health staff are to be notified and included in MDT meetings and risk assessment as appropriate.
- Providing meaningful activities and engagement to reduce boredom, distress, and feelings of confinement.
- Maintaining an appropriate level of observation by nursing staff, including increased supportive observations for people with concerns identified in their risk assessment.
- Orientation information to include the expectation that people receiving voluntary care will notify staff of their whereabouts if they plan to leave the inpatient unit.
- Considered and proportionate use of secure wards, guided by individualised risk assessments and clinical need. The use of secure wards should not be routine or punitive, but rather implemented only when necessary for the safety of the person receiving care and/or others.
- Granting leave for people receiving inpatient care when appropriate to do so.

Refer to the relevant service's leave-related policies, including:

- [AKG- Granting and Management of Leave for Mental Health Inpatients Policy](#)
- [RPBG- Leave for Mental Health Inpatients Policy](#)

- Encouraging people receiving care to let staff know if they have concerns about being on the ward.
- Addressing concerns around household and other responsibilities (e.g., related to caring for children, pets, etc).
- Promoting and supporting contact with family, friends and appropriate services as appropriate.
- Communicating potentially distressing information in a sensitive manner, for example news regarding family illness or death, legal issues, or changes to the person's leave or discharge plans. Staff must consider the person's current mental and emotional state and preferred communication style, and preferably deliver potentially distressing information in the presence of a trusted support person.
- Post-incident debriefing for both staff and people accessing care, to promote learning and continuous improvement.
- Ensuring environmental controls are implemented where appropriate, for example securing exits and considering the physical layout of the unit such as the location of rooms in relation to exits and the nursing station.

3.1 Preventing further harm after an incident occurs

When an incident (or near miss) occurs involving a person receiving care going AWOL or missing, immediate action must be taken. This includes, but is not limited to:

- Implementing any preliminary actions to mitigate further risk of harm;
- Ensuring the surrounding environment (physical and psychological) is safe to prevent immediate recurrence of the incident; and
- Debriefing with the people involved in the incident (including staff, the person receiving care upon return, and the person's carers and support people).

Refer to

- MP 0181/24 [Safety Planning for Mental Health Consumers Policy](#)
- Department of Health - [Best Practice Guideline: Mental Health Consumers Who Are Missing or Absent Without Leave](#)

4. Reporting and investigation obligations

All incidents (or near misses) where people accessing mental health services are AWOL or missing are to be notified via notification of the incident into the Datix Clinical Incident Management System (CIMS). This requirement meets the obligation to report these events as notifiable incidents to the Chief Psychiatrist as per [EMHS notifications requirements under the WA Mental Health Act 2014 Policy](#), [Policy for](#)

[Mandatory Reporting of Notifiable Incidents to the Chief Psychiatrist](#), and [MP 0122/19 Clinical Incident Management Policy](#).

When reporting the circumstances of incidents involving AWOL/missing persons from EMHS mental health services, Severity Assessment Code (SAC) ratings will be applied directly to the Datix-CIMS incident notification. The SAC rating will be determined on the basis of the Treating Consultant Psychiatrist's assessment of the level of clinical risk, both to the person directly and to the public, at the time the person was determined to be missing.

The Treating Consultant is responsible for the above assessment, be that person a Psychiatrist, ED Physician or other.

The assessment of clinical risk shall not be directly impacted by the person's existing status under the MHA 2014. It is acknowledged however, that people with elevated levels of clinical risk may have had this reflected in their legal status preceding the determination that they were AWOL or otherwise missing.

Staff must also document the AWOL or missing person incident in the person's medical record and as an alert/incident into PSOLIS.

4.1 Allocation of a SAC rating to an AWOL or Missing person clinical incident

In circumstances where the person receiving care and/or others have come to actual harm as a result of the clinical incident, the SAC rating will be allocated on the basis of whichever rating is higher:

- i. The rating corresponding to the highest level of harm experienced by the person receiving care and/or others.
- ii. The rating corresponding to the highest level of harm that could conceivably have been experienced by the person receiving care and/or others in that unique circumstance.

In circumstances where no actual harm was experienced as a result of the clinical incident, the SAC rating for the incident will be allocated on the basis of that person's clinical risk profile at the time leading up to when they were determined to be AWOL/missing. The assessment of a person's risk profile (i.e., risk concerns; no risk concerns) is based on their mental health condition and is determined using clinical judgement.

Table 1: Determination of SAC rating when reporting a missing person into Datix-CIMS

SAC 1	A clinical incident that has or could have resulted in serious harm or death (including near miss incidents); and which is attributed to health care provision (or lack thereof) rather than the patient's underlying condition or illness. When the patient outcome of a clinical incident is death and during review it is determined that there is any possibility that it was preventable, it must be notified as a SAC 1 and investigated as such. If a person deemed to have risk concerns goes missing from the health service, this should be notified as a SAC 1 clinical incident. People receiving involuntary care under the MHA 2014 who are AWOL are automatically deemed to have risk concerns and therefore should be notified as a SAC 1 clinical incident.
SAC 2	A clinical incident that has or could have resulted in moderate harm (including near miss incidents); and which is attributed to health care provision (or lack thereof) rather than the patient's underlying condition or illness.
SAC 3	A clinical incident that has or could have resulted in minor or no harm (including near miss incidents); and which is attributed to health care provision (or lack thereof) rather than the patient's underlying condition or illness.

Upgrading of the SAC rating following notification of the clinical incident is permissible if it becomes known that harm did occur, or was likely to occur (near miss), in accordance with the severity of that harm.

The level of investigation that is to occur in response to clinical incidents where people receiving care are AWOL or otherwise missing from their care setting is to correspond to the confirmed SAC rating applied by the Senior Staff member in the area in which the clinical incident occurred.

5. Compliance monitoring and legislative penalties

5.2 Legislative obligations

There are no legislative obligations relating to this Guideline for which a statutory penalty is imposed for a breach.

6. Legislation

- Mental Health Act 2014 (WA)
- Mental Health Regulations 2015 (WA)

6. Additional references

6.1 WA Health policies

- [MP 0012/16 Mental Health Consumers Who Are Missing or Absent Without Leave Policy](#)
- [MP 0012/16 Procedures for Involving Police When Mental Health Consumers Are Missing or Absent Without Leave](#)
- [Best Practice Guideline: Mental Health Consumers Who Are Missing or Absent Without Leave](#)
- [Triage to Discharge - Mental Health Framework for State-wide Standardised Clinical Documentation](#)
- [MP0122/19 Clinical Incident Management Policy](#)
- [Clinical Incident Management Guideline](#)
- [MP0155/21 Statewide Standardised Clinical Documentation for Mental Health Services](#)
- [MP 0181/24 Safety Planning for Mental Health Consumers Policy](#)
- [MP0063/17 Road-Based Transportation for Mental Health Consumers Policy](#)
- MP 0099/18 [Community Mental Health Status Assessments: Role of Mental Health Clinician Policy](#)
- [Notifications requirements under the WA Mental Health Act 2014](#)

6.2 EMHS policies

- [Care Coordination in Mental Health Services](#)
- [Patient Identification and Procedure Matching](#)

- [Reporting and Care of the Deceased](#)
- [Transportation of Mental Health Consumers](#)
- [Use of Video Monitoring in Mental Health Inpatient Units](#)
- Mental Health Missing Person Absent Without Leave

6.3 Other

- [Death in Hospital Form](#)
- [Chief Psychiatrist Clinicians' Practice Guide to the Mental Health Act 2014](#)
- [Policy for Mandatory Reporting of Notifiable Incidents to the Chief Psychiatrist](#)
- [Mosel, K. A., Gerace, A., & Muir-Cochrane, E. \(2010\). Retrospective analysis of absconding behaviour by acute care consumers in one psychiatric hospital campus in Australia. International journal of mental health nursing, 19\(3\), 177–185.](#)
- [Stokes, B \(2012\). Review of the admission or referral to and the discharge and transfer practices of public mental health facilities/services in Western Australia. Perth, Government of Western Australia, Department of Health and Western Australian Mental Health Commission](#)